

VIEWPOINT

The One Big Beautiful Bill Act—Making Medicaid and the Exchanges Sustainable

Anup Malani, PhD, JD

President Trump's working families tax cuts legislation (Public Law [PL]-119-21), commonly known as the One Big Beautiful Bill Act, aims to preserve the long-term feasibility of Medicaid and health insurance exchanges. During the COVID-19 pandemic, the federal government expanded Medicaid and premium subsidies on the exchanges. After the pandemic ended, these emergency benefits remained, although they are financially unsustainable for an aging population. PL-119-21 acknowledges this concern and reduces spending growth to rates supported by Democrats as recently as 2019.

Despite these facts, many argue¹ that PL-119-21 guts Medicaid and the Patient Protection and Affordable Care Act (ACA). That view is inconsistent with statistics on Medicaid enrollment and expenditures and with the text of the ACA. A fair analysis of PL-119-21 demonstrates that it is an effort to achieve common-sense management of spending on Medicaid and the exchanges.

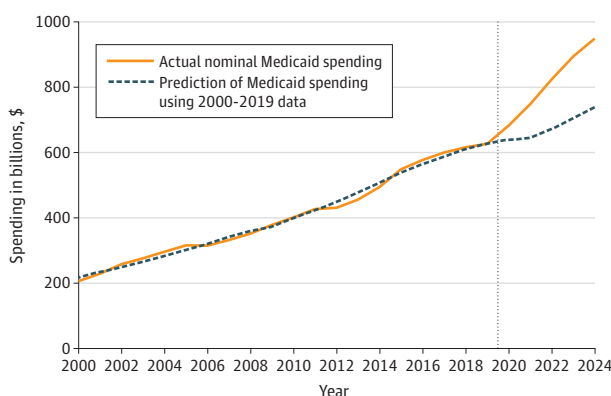
Medicaid

PL-119-21 restores Medicaid growth to historical levels supported by Democrats under President Obama. From 2000 to 2019, Medicaid spending grew 6.3% annually, even under the ACA. When the pandemic began, annual spending growth increased to 8.7% (Figure). In 2024, Medicaid spent \$210 billion more than it would have (\$739 billion) had its growth rate not risen in 2020.

Medicaid spending accelerated because of both additional enrollment and higher reimbursements for providers. Enrollment spiked after the Families First Coronavirus Response Act (FFCRA, 2020) incentivized states to continuously enroll beneficiaries and provided an option to relax eligibility requirements. According to my calculations, a higher rate of enrollment growth accounted for approximately 60% of Medicaid spending growth from 2020 to 2024, with most growth concentrated in 2020 to 2022. Unfortunately, additional Medicaid enrollment does not always reduce the number of uninsured individuals: 21% of Medicaid enrollees also have private insurance coverage² and, according to the Medicaid and CHIP Payment and Access Commission (October 2021), 4% to 6% simultaneously had Medicaid coverage in multiple states even before FFCRA.

After 2022, enrollment growth slowed, but spending growth continued due to higher payment rates. Medicaid is jointly funded by the federal government and states but is administered by states. This misalignment gives providers and states an incentive to modify Medicaid to increase the federal government's share of its financial burden. During the pandemic, hospitals lobbied states to raise provider reimbursements via state-directed payments (SDPs). The Office of the Actuary Centers for Medicare & Medicaid Services (CMS) estimated that SDP payments exceeded \$97.8 billion in 2024 alone, approximately

Figure. Recent Medicaid Spending Compared With Predicted Medicaid Spending Based on 2000 to 2019 Data



The solid orange line provides actual nominal Medicaid spending (federal plus state), using data from the Centers for Medicare & Medicaid Services Office of the Actuary (OACT). The dashed line presents a prediction of nominal Medicaid spending obtained from a linear regression (conducted by the author) of log spending (OACT) on log population (US Bureau of Economic Analysis), log employment rate (Federal Reserve Economic Data [FRED]), log unemployment rate (FRED), and log medical Consumer Price Index (FRED). The regression was estimated using only 2000-2019 data, but the prediction used data from 2000-2024. The adjusted R^2 is 0.9761. The 2000-2019 segment of the dashed line shows that the prediction model fits the actual pre-2019 data well. The 2020-2024 segment presents a baseline (ie, what would have happened to Medicaid spending in the absence of the COVID-19 pandemic).

half the additional 2024 spending due to faster spending growth starting in 2020. In some cases, SDPs caused Medicaid to pay hospitals up to 2.5 times what they were paid under Medicare.³ States covered their share of SDPs by raising taxes on providers. These moves raised hospital revenues, but primarily by increasing federal spending.

PL-119-21 reins in the acceleration of Medicaid spending in 2 ways. First, it manages enrollment growth by requiring Medicaid eligibility checks for adults twice annually and limiting retroactive coverage of pre-enrollment medical expenses. It limits enrollment of non-citizens, deceased persons, and persons enrolled in Medicaid in multiple states.

PL-119-21 adds community engagement requirements to Medicaid. Individuals may satisfy them by working, taking classes, or volunteering. Vulnerable groups—individuals with disabilities or child-care responsibilities, tribal members, and elderly individuals—are exempt. Covered individuals must participate only part time (80 hours per month) in qualifying activities. PL-119-21 budgets \$200 million to help enrollees navigate this requirement. CMS is using this funding to build a digital verification system all states can use to reduce the hassle of demonstrating compliance and, where possible,

help individuals find activities to satisfy community engagement requirements. This effort should be viewed not as a cut to Medicaid but as a program to help able-bodied, working-age adults (18-64 years) gain skills and work toward financial independence.

Second, PL-119-21 slows growth in spending per enrollee. It limits SDPs so that Medicaid reimburses providers at approximately the level Medicare does, closing a corporate rift. Paying hospitals more under Medicaid than Medicare lacks economic logic and unjustly values older people who paid into Medicare their entire lives less than those who have Medicaid.

PL-119-21 gradually lowers provider taxes to ensure that the federal government and states share the cost of Medicaid. Historically, the federal government covered almost 60% of Medicaid costs. The ACA raised that amount to 90% for new, able-bodied, adult enrollees to encourage states to expand Medicaid. PL-119-21 simply ensures that the states do not use excessive provider taxes to evade this cost-sharing agreement. States could join the reform effort by reducing fraud, waste, and abuse; right-sizing reimbursement rates; or managing local revenues if cost containment is insufficient. To help, PL-119-21 provides states \$50 billion to support rural health care.

Casting the PL-119-21 reforms as a cut to Medicaid is a mathematical sleight of hand. Under the act, Medicaid spending—even per enrollee—is projected to increase. The CBO estimated that federal Medicaid spending will grow during the next 10 years under PL-119-21 but at a rate that is 1.4 percentage points⁴ lower. In short, the act returns spending to historic and sustainable rates.

Health Insurance Exchanges

PL-119-21 has a narrow effect on health insurance exchanges. First, it is the Inflation Reduction Act of 2022, not PL-119-21, that sunsets (in 2025) the enhanced premium subsidies that the American Rescue Plan Act provided during the pandemic.

Second, the focus of PL-119-21 is eliminating wasteful federal spending on the exchanges. Previously, individuals were automatically reenrolled in their insurance plan, sometimes without their knowledge. Last year, the government paid for insurance for nearly 12 million persons who never used their coverage. It is estimated that 6.4 million enrollees either were ineligible for coverage or did not know they had coverage and thus did not use it.⁵ PL-119-21 eliminates these so-called phantom patients by requiring income verification before enrollment.

Conclusion

Although critics focus on the number of people who will lose coverage under PL-119-21, those numbers mask 2 details. First, the losses are calculated by subtracting projections of coverage during 10 years under PL-119-21 from projected coverage without PL-119-21. Although coverage under PL-119-21 is projected to be lower than coverage without the act, the CMS Office of the Actuary projects enrollment to increase over time even under PL-119-21. Second, the total enrollment numbers ignore who would lose coverage even as net enrollment grows. Of individuals losing Medicaid coverage (4.76 million by 2027), approximately 1.4 million are undocumented immigrants and 1.3 million were already ineligible under prior law.

PL-119-21 attempts to find common ground. It restores Medicaid spending growth and allows the premium subsidy schedule to return to levels President Obama supported in the ACA. The PL-119-21 community engagement requirements are modeled on reforms from the 1990s that President Clinton advocated as “a crusade to...transform our system of dependence into a system of independence; [and] to prove that poor people could succeed.”⁶ Finally, the act ensures that Medicaid and the exchange subsidies will be financially feasible in a decade so our nation can afford to support individuals who fall on hard times in the next generation.

ARTICLE INFORMATION

Author Affiliations: Centers for Medicare & Medicaid Services, Washington, DC; Law School, University of Chicago, Chicago, Illinois.

Corresponding Author: Anup Malani, PhD, JD, Centers for Medicare & Medicaid Services, 200 Independence Ave SW, Washington, DC 20201 (anup.malani@cms.hhs.gov).

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